Georgia's Naloxone Access Policy and the Opioid Crisis in Cobb County

Adrien Jefferson

Georgia Highlands College

Health Professions

Sharon Bailey

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Over the last several years, the opioid crisis has become impossible to ignore in the United States. News reports, public health data, and even personal stories show how serious the issue has become. Georgia has experienced this firsthand. As overdose deaths increased, the state expanded access to naloxone, a medication that can reverse an opioid overdose if given quickly. By allowing pharmacists to dispense naloxone without an individual prescription and by strengthening Good Samaritan protections, Georgia tried to make it easier for people to step in during emergencies. The goal was simple: reduce deaths by removing hesitation and legal fear when someone needs help.

Policy Background and Problem Addressed

The policy was responding to a growing problem. Opioids include prescription painkillers, heroin, and synthetic drugs such as fentanyl. Over time, misuse of these substances has led to a sharp increase in overdose deaths. Recent data from the CDC (2023) show that drug overdoses are still among the top causes of injury-related deaths in the United States, with synthetic opioids accounting for the majority of those cases. Georgia's numbers follow the same general trend. The Georgia Department of Public Health (GDPH, 2023) continues to report increases in opioid-related emergency department visits and deaths, especially in metro Atlanta areas. Fentanyl has made the situation even more dangerous because even small amounts can be deadly.

One thing that stands out to me is that opioid exposure does not always begin with illegal drug use. Many people first encounter opioids through prescriptions meant to treat real pain. That part feels especially relevant to me because I plan to work in sports medicine. Athletes often deal

with serious injuries. In some cases, opioids may be prescribed after surgery or significant trauma. While that can be medically appropriate, longer use or higher doses can increase the risk of misuse. That connection between injury treatment and possible long-term consequences makes the opioid crisis more than just a public health statistic. It becomes a clinical issue that health care providers need to think carefully about.

Stakeholders and Implementation Process

Expanding access to naloxone was not something one group handled on its own. It took effort from different parts of the state. Lawmakers passed laws to make naloxone easier to get and to protect people who try to help during an overdose. The Georgia Department of Public Health keeps track of overdose data and helps organize how naloxone is distributed. Because of the statewide standing order, pharmacists can give out naloxone without someone needing a personal prescription, which makes it easier for people to access it. First responders usually carry naloxone and use it in emergencies. Community organizations also help by handing out kits and teaching people what to look for if someone is overdosing. Doctors and other healthcare providers play a part by being careful about how they prescribe opioids and by talking to patients about the risks. At the end of the day, the people most affected are those dealing with opioid addiction and the families trying to support them.

Putting the policy into action required several concrete changes. Good Samaritan protections were strengthened so people would feel safer calling 911 if someone was overdosing. The state also issued a standing order that lets pharmacists provide naloxone without an individual prescription, which made access more direct. Public health departments worked to expand availability, especially in areas with higher overdose rates. At the same time, training

programs helped both first responders and community members learn how to spot the signs of an overdose and act quickly. Outreach campaigns also aimed to reduce the stigma that often surrounds substance use. Overall, these efforts were designed to make emergency response faster, easier, and more effective.

Policy Impact

Research suggests that expanded naloxone access has helped. Studies show that take-home naloxone programs are linked to lower overdose death rates (McDonald & Strang, 2016). The CDC (2023) identifies naloxone distribution as an effective strategy for preventing fatal overdoses. Georgia's reports show increased distribution and documented overdose reversals (GDPH, 2023). At the same time, overdose deaths remain high, largely because fentanyl continues to circulate widely. This shows that while naloxone can prevent death in an emergency, it does not solve the larger issue of opioid misuse. It is a critical response tool, but it is not a complete answer.

Current Public Health Challenge in Cobb County

In Cobb County, opioid misuse continues to affect the community. National data from the CDC (2023) show that synthetic opioids account for most overdose deaths in the United States, and Georgia data show similar trends (GDPH, 2023). The impact is felt across age groups, especially among working-age adults. It also affects families, healthcare systems, and emergency services. In communities where youth and collegiate sports are a big part of daily life, this issue often ties back to how injuries are treated and how pain medication is prescribed. The CDC's 2022 Clinical Practice Guideline for Prescribing Opioids for Pain encourages healthcare providers to consider non-opioid options first and to limit opioid use whenever it is appropriate

to do so (Dowell et al., 2022). These recommendations highlight how prevention can begin in clinical settings.

Proposed Evidence-Based Intervention

An evidence-based approach for Cobb County would be strengthening safe prescribing practices in sports medicine and orthopedic care. Pain can often be managed through physical therapy, rehabilitation methods, and non-opioid medications. Educating patients about risks before prescribing opioids can also reduce misuse. Instead of focusing only on reversing overdoses after they occur, this approach tackles potential risk earlier in the process.

As a future sports medicine physician, I see this as part of professional responsibility. Helping an athlete recover is important, but so is protecting their long-term health. Safe prescribing and patient education are not just clinical details. They are part of a larger effort to reduce harm in the community. Georgia's naloxone access policy plays a vital role in emergency response. However, long-term progress depends on combining emergency measures with prevention strategies built into everyday healthcare practice.

Conclusion

Georgia expanded naloxone access in an effort to lower overdose deaths and make it easier for people to step in during emergencies. Evidence shows that having naloxone more widely available does prevent fatalities, even though overdose rates remain high, largely because of synthetic opioids like fentanyl. In Cobb County, opioid misuse continues to affect individuals and families across the community. Focusing on safer prescribing practices in sports medicine is a realistic and research-backed way to reduce risk earlier, especially for patients recovering from

injuries. Tackling the opioid crisis takes more than one solution. It requires quick action in emergencies and stronger prevention efforts over time, and healthcare providers have a responsibility to support both.

References

Centers for Disease Control and Prevention. (2023). *Drug overdose deaths in the U.S.* https://www.cdc.gov/drugoverdose/deaths/

Dowell, D., Ragan, K. R., Jones, C. M., Baldwin, G. T., & Chou, R. (2022). CDC clinical practice guideline for prescribing opioids for pain — United States, 2022. *MMWR Recommendations and Reports, 71*(3), 1–95. https://doi.org/10.15585/mmwr.rr7103a1

Georgia Department of Public Health. (2023). *Drug surveillance unit: Opioid data and surveillance reports.* https://dph.georgia.gov

McDonald, R., & Strang, J. (2016). Are take-home naloxone programmes effective? Systematic review. *Addiction, 111*(7), 1177–1187. https://doi.org/10.1111/add.13326